A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Timothy Chew
Turn The Corner Medical Clinic

Dear Dr Chew,

Re: Miss Zoe Cosker
DOB: 1987-07-27
Address: 402/85 Hutton Street, THORNBURY, VIC, 3071

Summary

- **Serrated polyposis syndrome with sessile serrated adenomas and hyperplastic polyps.**
- **Recurrent anal fissure causing significant post-defecatory pain.**
- **Commencement of low-dose enteric-coated aspirin for polyp prophylaxis.**
- **Annual surveillance colonoscopy required.**
- **No action required by you.**

I had the pleasure of conducting a review with Zoe to discuss her recent colonoscopy findings and a new exacerbation of anal pain. Zoe reported significant discomfort during defecation, with pain persisting for up to eight hours following a bowel movement. Given her clinical history, this is highly suggestive of a recurrent anal fissure. Her bowel movements are currently formed and stable, which appears to be contributing to the mechanical strain during evacuation.

The recent colonoscopy revealed a combination of sessile serrated adenomas and hyperplastic polyps. Histologically, there was no evidence of high-grade dysplasia. However, the number and nature of these polyps confirm a diagnosis of serrated polyposis syndrome. We discussed the clinical implications of this condition, including the necessity for frequent monitoring to prevent the development of colorectal malignancy. We also discussed the potential benefits of long-term aspirin therapy in reducing the recurrence of serrated polyps. While Zoe has a previous history of moderate gastritis, she is comfortable proceeding with a trial of low-dose, enteric-coated aspirin. I have counselled her on the importance of monitoring for any symptoms of upper gastrointestinal irritation

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan

Zoe has been provided with a prescription for compounded nifedipine and lignocaine cream to treat the anal fissure. I have advised her to apply this at least twice daily for a period of four to six weeks to ensure complete healing. To further reduce internal anal sphincter pressure, I have recommended the use of a footstool to achieve a squatting position during defecation and the regular consumption of kiwi fruit as a natural stool softener.

Regarding her serrated polyposis syndrome, Zoe will commence enteric-coated aspirin 100 mg daily. I have recommended an annual surveillance colonoscopy, with the next procedure due in twelve months. Zoe also mentioned an upcoming permanent residency application; I have informed her that I am happy to provide a clinical summary to support her application if required.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

COSKER, ZOE THOMSON : SVP 893507

DOB: 27/07/1987 Age: 38 years

Unit 402, 85 Hutton St, Thornbury 3071

Results from 11/04/2026 ..... Episode# 26P258063 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009376 **CLINICAL NOTES** SSPS. SSA's. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Ascending Colon Polyp". Multiple polypoidal mucosal fragments measuring 7 x 4 x 3 mm in aggregate. A1. 2. Specimen labelled "Transverse Colon Polyp". Two tan polypoid tissus 19 x 5 x 1 mm to 22 x 6 x 1 mm. Margin inked blue. Also received multiple polypoidal

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mucosal fragments measuring 11 x 6 x 2 mm in same container. Block details: 2A:7TS. 2B : 7 TS. 2C: Multiple polypoid fragments processed whole. A3. 3. Specimen labelled "Sigmoid Polyp". Eight tan polypoid fragments measuring 3 x 2 x 1 mm to 6 x 3 x 2 mm. Processed whole. A1 **MICROSCOPIC DESCRIPTION And DIAGNOSIS** 1. Ascending colon polyp - Hyperplastic polyp. No evidence of dysplasia or malignancy. 2. Transverse colon polyp - Sessile serrated lesion with no evidence of dysplasia or malignancy. 3. Sigmoid polyp - Hyperplastic polyp. No evidence of dysplasia or malignancy. Pathologist: Dr Cesar Salinas-LaRosa SNOMED T#: 63 SNOMED M#: 00400

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